

Using Your Inhaler Correctly

A patient guide for pressurized metered-dose inhalers (pMDI) for asthma and COPD

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Why does correct inhaler technique matter?

Inhalers are the cornerstone of treatment for asthma and chronic obstructive pulmonary disease. Because you breathe the medication directly into your lungs, it works quickly and at the site where it is needed.

However, when the technique is wrong, more than half of the medication can stay in your mouth and throat — never reaching the lungs. International research (Sanchis 2016, CRITIKAL 2017) shows that approximately 70% of inhaler users make at least one important error.

Three errors in particular are severe enough to invalidate the dose entirely:

Pressing the canister BEFORE starting to breathe in — the medication ends up in the mouth, almost none reaches the lungs

Breathing out into the inhaler — the dose is dispersed or contaminated

Not holding your breath after inhaling — the medication leaves the lungs before it can settle

Correct inhaler technique is as important as the prescription itself. Mastering the 7 steps on the next pages will help you get the full benefit of your medication.

Correct pMDI Use — 7 Steps

Practice each step once in front of a mirror. It feels awkward at first, but becomes natural after a few tries.

1

Shake the inhaler

Hold the inhaler firmly and shake it up and down 4 to 5 times to mix the medication evenly.

Tip: A vigorous shake ensures the medication mixes well inside the canister.

2

Remove the cap and inspect

Take off the mouthpiece cap and look inside to make sure there is no debris or obstruction.

Tip: Small particles can occasionally fall in between uses.

3

Exhale slowly, away from the inhaler

Hold the inhaler away from your face and breathe out fully and gently. Do NOT blow into the mouthpiece.

Tip: You need to empty your lungs so the medication has room to enter.

4

Place mouthpiece in mouth and seal lips

Place the mouthpiece between your teeth and seal your lips tightly so no air leaks. Keep your tongue flat so it does not block the mouthpiece.

Tip: Lips should be firm enough that no air escapes around the mouthpiece.

5

Press canister AS you begin inhaling

Start a slow, deep breath in — and at the very moment you start, press the canister once. This is the most important step.

Tip: Not 'inhale then press' but 'press while inhaling'.

6

Continue slow, deep inhalation

Keep breathing in slowly and deeply for 4 to 5 seconds after pressing, filling your lungs completely.

Tip: Like inflating a balloon slowly, not one quick gulp.

7

Hold breath for 10 seconds, then exhale slowly

Remove the inhaler from your mouth and hold your breath for about 10 seconds (count slowly to 10). Then exhale gently through pursed lips.

Tip: The longer you hold, the more medication deposits in the lungs.

If you use an inhaled corticosteroid (ICS)

After using a steroid-containing inhaler (e.g., Pulmicort, Flixotide), rinse your mouth with water and spit out. Residual medication in the mouth and throat can cause thrush (a fungal infection) or hoarseness.

Frequently Asked Questions

Q. I don't feel the effect right away. Can I use it again?

A. Stick to the prescribed dose and interval. Some medications take time to act, and extra doses can increase side-effect risk. If you feel the effect is inadequate, speak with your clinician before adding doses.

Q. Do I need to shake the inhaler every time?

A. Most metered-dose inhalers require shaking before use. Some solution-based inhalers do not — check your specific device leaflet.

Q. Do I need a spacer?

A. Spacers are strongly recommended for children, the elderly, or anyone who has difficulty coordinating inhalation. They help more medication reach the lungs and reduce side effects (hoarseness, oral candidiasis).

Q. How do I check how many doses are left?

A. Modern pMDIs usually have a counter on top. For inhalers without a counter, track manually based on prescription date and daily use. The 'float test' in water is unreliable and not recommended.

Q. I'm worried I've been doing it wrong.

A. Don't worry — inhaler technique is rarely perfect the first time. Your clinical team will check together at each visit. Even if your evaluation is not perfect, consistent practice is what matters most.

Finally

Inhaler technique does not have to be perfect from the start. Working step by step with your healthcare team, you will reach the point where correct technique feels natural. Do not be discouraged by less-than-perfect evaluation results — focus on the one or two steps that need improvement. Small improvements add up to better lung health. We are with you.